EARLY DETECTION AND CONTROL OF LIPIDS TO TARGETS ACCORDING TO RISK IS THE WAY TO REDUCE HEART ATTACK

India Braces Up to Combat the 'Real Silent Killer'

Cardiovascular diseases occur at a younger age in Indians with more than 50% are getting heart attack below 50 years. The main culprit in most cases is dyslipidemia, the situation of abnormally elevated lipid levels in the blood and hence early detection and treatment of the cause is vital. But until a few months ago, India lacked country specific guidelines to deal with lipids and its associated diseases. That was when the Cardiological Society of India rose to the occasion and initiated a process to draft the clinical practice guidelines for dyslipidemia management by a group of top cardiologists of India led by Dr. J P S Sawhney, Chairman of Cardiology at Sir Ganga Ram Hospital, Delhi. Our reporter caught up for a tete-a-tete with Dr Sawhney to discuss the findings of the expert panel and the way forward for lipid management in India.

Q. Dr. Sawhney, first of all congratulations. Before we get into the details of the new guidelines, we must know from you what necessitated the need for a India specific dyslipidemia management guideline? Are we different from rest of the world?

In our country 25% to 30% population die of cardiovascular diseases. Yes, Indian population is different from the western world, as we get the coronary artery disease almost 10 years earlier. The main cause of this disease is the mismanagement of lipids in our bodies and there is lack of proper guidelines for dyslipidemia management. Indians acquire risk factors like hypertension and diabetes, early in life. Central obesity as a component of metabolic syndrome is much more common in our population. Beedi/Chewing tobacco is also rampant (possibly restricted to India) and is also one of the causes for early heart attacks.

India has a high prevalence of atherogenic dyslipidemia which is characterised by high Triglycerides (TGY), low HDL and small dense LDL-cholesterol. High prevalence of heterozygous familial hypercholesterolemia (HeFH) and elevated Lipoprotein(a) (Lp(a)) among Indians as compared to western world are the specific genetic disorders of Lipids in Indians. Sedentary lifestyle and unhealthy cooking and high consumption of carbohydrate diet are some other factors which puts the Indian population in a more risky situation.

Hence, there was an urgent need to have a country specific guideline in place to suit our unique ethnic, genetic and socio-cultural fabric.

Q. We are interested to know the roadmap of the guidelines which the expert group of cardiologists lead by you have come up with, is it based on research on the Indian population?

Yes, it took us more than 4 years along with 22 cardiologists who are experts in this field to prepare this document. It is customised to the Indian population. We had good data on epidemiology of Lipids, clinical trials where our population participated, genetic studies of FH and Lp(a) and response to antihyperlipidemic drugs in a small number of patients of our country.

However, many hard core data were not available so we referred to the widely accepted international guidelines of the European Society of Cardiology (2019) and combined it with

